

WEEK 9 — BUSINESS TRANSFORMATION (B2O)

# Turning Around an Underperforming Hospital Group

*Diagnosing a business-model / operating-model misalignment across three hospitals, and designing the transformation plan to fix it.*



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Prepared for: Hospital Group Leadership | Riyadh, KSA

*Continuation of Week 8 Performance Diagnosis | New Engagement: Al-Noor Hospital Group*

# “The Market Is Competitive” Is Not a Diagnosis

*Leadership's working theory explains away the symptom. Our job is to test it — and find out whether the real problem is strategy, operations, or both.*

1

## Not just financial analysis

We use 3 years of actuals as evidence — but the deliverable is a root-cause diagnosis and a transformation plan, not a bigger spreadsheet.

2

## Business Model vs. Operating Model

Every finding is sorted into “what the hospital sells and to whom” (B) versus “how it actually delivers and runs” (O).

3

## One clear answer

The business is underperforming because X, and we recommend Y — stated once, in one line, at the end.

### SCOPE

- 3 hospitals, 1 group, Riyadh, KSA
- 3-year trailing quarterly-equivalent financials: Revenue, OpEx, Operating Profit, Margin
- Driver-based diagnostic — pattern recognition on existing data, not a full re-forecast
- B2O lens: Business Model (what/who/pricing) vs. Operating Model (people/process/tech/governance)
- Output: root-cause diagnosis + prioritized transformation roadmap

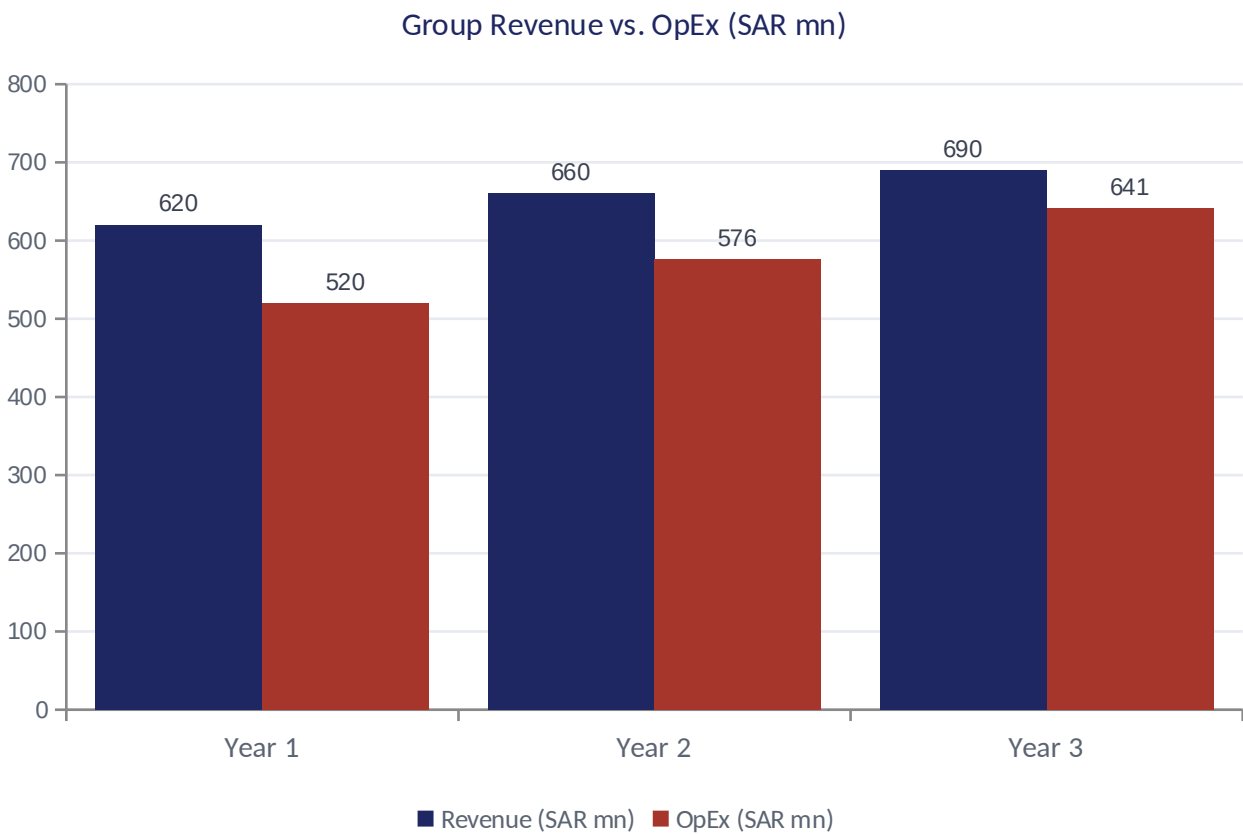
# Al-Noor Hospital Group: Three Hospitals, One P&L

A mid-sized Riyadh group, ~440 beds across three sites with very different positioning — consolidated reporting has been hiding that difference.

|                                                                                                                                                                                                                    |                                                                                                                                                                                                                            |                                                                                                                                                                                                     |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <div>Al-Noor Central</div> <div>General • Mid-Market • Legacy Flagship</div> <div>220 beds</div> <div>The group's original hospital — broad general-medicine service line, oldest systems and org structure.</div> | <div>Al-Noor Premium Ortho &amp; Spine</div> <div>Specialty • Premium • Newest Build</div> <div>90 beds</div> <div>Purpose-built specialty hospital, opened most recently, protocolized orthopedic &amp; spine care.</div> | <div>Al-Noor East</div> <div>Community • Mid-Market • Growth Site</div> <div>130 beds</div> <div>Added to the group to capture east-Riyadh demand growth — expanded fastest, resourced least.</div> |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|

**Leadership's view:** “The market is competitive — that's why margins are under pressure.” Our task: test that against three years of actuals.

# Revenue Is Growing. Costs Are Growing Faster.



**+11.3%**

Cumulative revenue growth  
(SAR 620mn → 690mn)

**+23.3%**

Cumulative OpEx growth  
(SAR 520mn → 641mn)

**-9.0 pts**

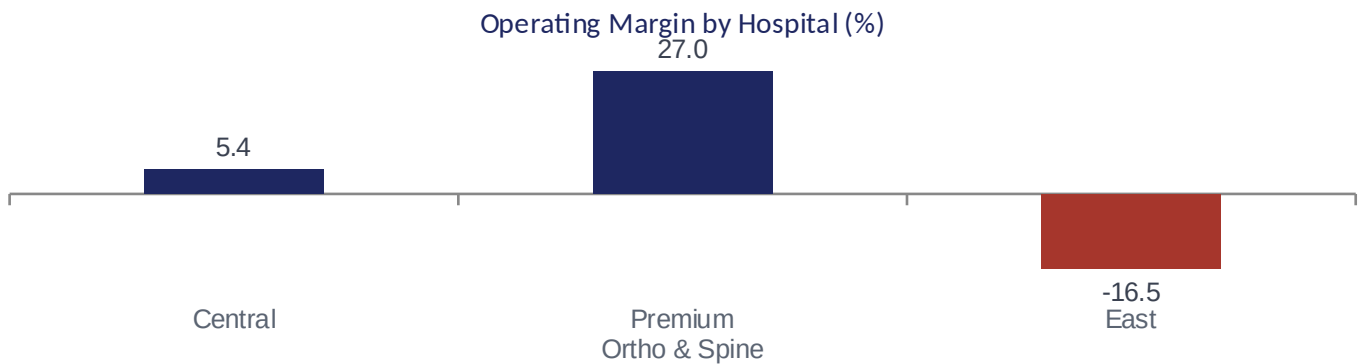
Operating margin lost  
(16.1% → 7.1%)

*Pattern: OpEx has grown at ~2x the rate of revenue in every year of the trailing window — this is structural, not a one-off.*

# The Group Average Is Hiding a 43-Point Margin Spread

Year 3 performance, by hospital. The consolidated 7.1% margin is not what any single hospital is actually experiencing.

| Hospital                      | Revenue (SAR mn) | OpEx (SAR mn) | Operating Margin |
|-------------------------------|------------------|---------------|------------------|
| Al-Noor Central               | 355              | 336           | 5.4%             |
| Al-Noor Premium Ortho & Spine | 196              | 143           | 27.0%            |
| Al-Noor East                  | 139              | 162           | -16.5%           |
| Group Total                   | 690              | 641           | 7.1%             |



READ

Al-Noor Premium Ortho & Spine is healthy — a 27% margin, in line with a well-run premium specialty hospital.

Al-Noor Central is thin but positive — 5.4% margin on a large revenue base signals a cost problem, not a demand problem.

Al-Noor East is loss-making — -16.5% margin means every admission currently destroys value.

# Three Different Businesses, Reported as One

What each hospital sells, to whom, and at what price point — the group's positioning is more varied than its reporting suggests.

|                   | Al-Noor Central                                     | Premium Ortho & Spine                             | Al-Noor East                                  |
|-------------------|-----------------------------------------------------|---------------------------------------------------|-----------------------------------------------|
| Positioning       | General, mid-market                                 | Specialty, premium                                | Community, mid-market                         |
| Target segment    | Broad local population, insured & self-pay          | Cash-pay / premium insurance, implant-heavy cases | East-Riyadh growth catchment                  |
| Avg. price / case | SAR 14,200                                          | SAR 40,000                                        | SAR 10,000                                    |
| Year 3 revenue    | SAR 355mn (51%)                                     | SAR 196mn (28%)                                   | SAR 139mn (20%)                               |
| Value proposition | Clear, but priced like a commodity general hospital | Clear and coherent — pricing matches service      | Unclear — built for growth, priced for volume |

The business model itself is coherent for all three hospitals. The open question is whether each hospital's operating model matches the business it's meant to run.

# How Each Hospital Actually Runs, Across Four Lenses

| LENS                                         | CENTRAL                                                     | PREMIUM ORTHO & SPINE                                | EAST                                                                 |
|----------------------------------------------|-------------------------------------------------------------|------------------------------------------------------|----------------------------------------------------------------------|
| <div><div>P</div><div>People</div></div>     | Large permanent headcount + heavy corporate/admin layer     | Lean clinical staffing, low agency reliance          | Chronically understaffed — plugged with high-cost agency/locum staff |
| <div><div>P</div><div>Process</div></div>    | Manual, paper-based approvals; slow patient flow            | Protocolized ERAS pathways; high bed turnover        | Ad hoc scheduling; frequent bottlenecks at admission & discharge     |
| <div><div>T</div><div>Technology</div></div> | Legacy hospital information system, limited digitization    | Modern scheduling & clinical systems                 | Same legacy system as Central, under-resourced IT support            |
| <div><div>G</div><div>Governance</div></div> | Centralized, slow decision rights; multiple sign-off layers | Delegated operational authority to hospital director | Decisions routed through group corporate — slow to resource          |

Where inefficiency lives: bloated overhead layers at Central, and a capacity gap plugged with expensive temp labor at East.  
Week 9 — Turning Around an Underperforming Hospital Group

# What's Actually Broken

## **Cost is structurally outpacing revenue**

OpEx grew +23.3% over 3 years vs. +11.3% revenue — roughly 2x, group-wide, every year.

## **The group average is masking the real problem**

A 7.1% consolidated margin hides a 27.0% performer and a -16.5% loss-maker sitting side by side.

## **Al-Noor Central is carrying legacy overhead it never shed**

Admin & corporate overhead is 15% of OpEx at Central vs. ~7% at the other two hospitals.

## **Al-Noor East scaled demand without scaling permanent capacity**

Agency/locum staffing is 26% of East's OpEx — growth funded by the most expensive form of labor available.

## **Al-Noor Premium Ortho & Spine proves the group can do this well**

Its 27% margin shows a coherent, well-run hospital is achievable within the same group.



# Al-Noor Central: High Cost Structure, Mid-Market Pricing

## MISALIGNMENT #1

Central prices and positions itself as a general, mid-market hospital — but it still carries the corporate overhead of a much larger, more complex organization. The operating model never shrank to fit the business model.

**BUSINESS MODEL (B)**  
  
General hospital. Broad local population. Priced at SAR 14,200/case — a mid-market point that leaves little room to absorb overhead.

**OPERATING MODEL (O)**  
  
Bloated corporate/admin layer, manual paper-based approvals, legacy IT — cost structure sized for a premium operation, not a mid-market one.



# Al-Noor East: Growth Strategy, Limited Capacity

MISALIGNMENT #2

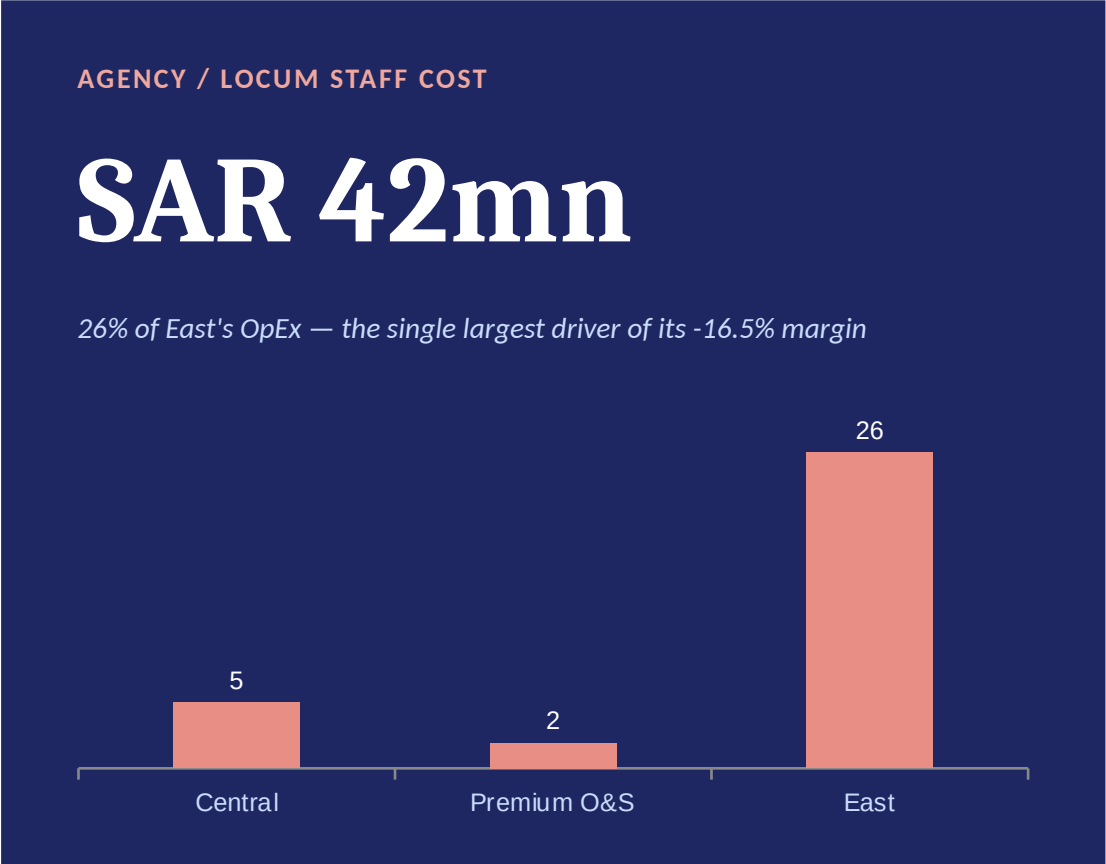
East was added to the group to capture east-Riyadh demand growth. Demand arrived — but permanent clinical staffing never scaled with it, so the hospital plugs the gap with agency and locum staff at a steep cost premium.

**BUSINESS MODEL (B)**

Community, mid-market hospital chasing a genuine growth catchment. Volume grew as planned, at SAR 10,000/case.

**OPERATING MODEL (O)**

Chronic understaffing forces reliance on agency/locum labor; ad hoc scheduling creates admission and discharge bottlenecks.



# Al-Noor Premium Ortho & Spine: What Good Looks Like

## ALIGNMENT BENCHMARK

The same group already runs one hospital where the operating model matches the business model. Premium pricing is backed by a lean, protocolized operation — proof that the fix is executional, not structural.

### BUSINESS MODEL (B)

Premium specialty hospital.  
Implant-heavy orthopedic & spine cases. Priced at SAR 40,000/case, aimed at a cash-pay / premium-insurance segment.

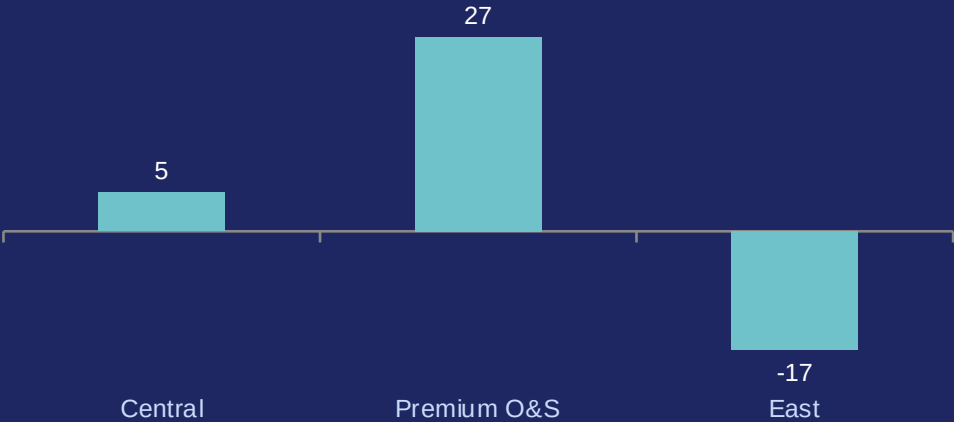
### OPERATING MODEL (O)

Lean clinical staffing, ERAS protocolized pathways, modern scheduling systems, delegated operational authority — low agency reliance, high bed turnover.

## OPERATING MARGIN

27.0%

Best performer in the group — the internal benchmark for the transformation



# Not the Market. The Match Between What We Sell and How We Run It.

*“The group is underperforming because two of its three hospitals — Al-Noor Central and Al-Noor East — are running an operating model that doesn't match their business model: legacy overhead priced for mid-market, and growth volume staffed with the most expensive labor available. Al-Noor Premium Ortho & Spine proves the fix works within this group today.”*

## Central

High-cost structure  
+ mid-market pricing

## East

Growth strategy  
+ limited capacity

## Premium O&S

Premium pricing  
+ lean operations = works

# Keep the Portfolio Strategy. Fix the Operating Model.

The three-tier business model doesn't need to change — the operating model at two of the three hospitals does.

| Al-Noor Central                                                                                                                                                                                                                                                             | Premium Ortho & Spine                                                                                                                                                                                                                                      | Al-Noor East                                                                                                                                                                                                                                                               |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <div><div>BUSINESS MODEL</div><div>Stays general, mid-market</div></div> <div><div>OPERATING MODEL — FROM</div><div>Bloated admin layer, manual approvals</div></div> <div><div>OPERATING MODEL — TO</div><div>Delayed corporate structure, digitized approvals</div></div> | <div><div>BUSINESS MODEL</div><div>Stays premium specialty</div></div> <div><div>OPERATING MODEL — FROM</div><div>Already lean and protocolized</div></div> <div><div>OPERATING MODEL — TO</div><div>Standardize its playbook across the group</div></div> | <div><div>BUSINESS MODEL</div><div>Stays community growth site</div></div> <div><div>OPERATING MODEL — FROM</div><div>Agency/locum-dependent staffing</div></div> <div><div>OPERATING MODEL — TO</div><div>Permanent variable-staffing model matched to demand</div></div> |

# From Diagnosis to Execution

1

## Quick Wins

0–6 months

- Begin delayering Central's corporate/admin structure
- Launch group-wide procurement RFP for medical supplies & implants
- Freeze further agency/locum hiring at East pending staffing plan

2

## Medium-Term

6–18 months

- Digitize Central's approval workflows end-to-end
- Recruit and onboard East's permanent clinical staffing pool
- Roll out Premium O&S's scheduling & ERAS protocols to Central

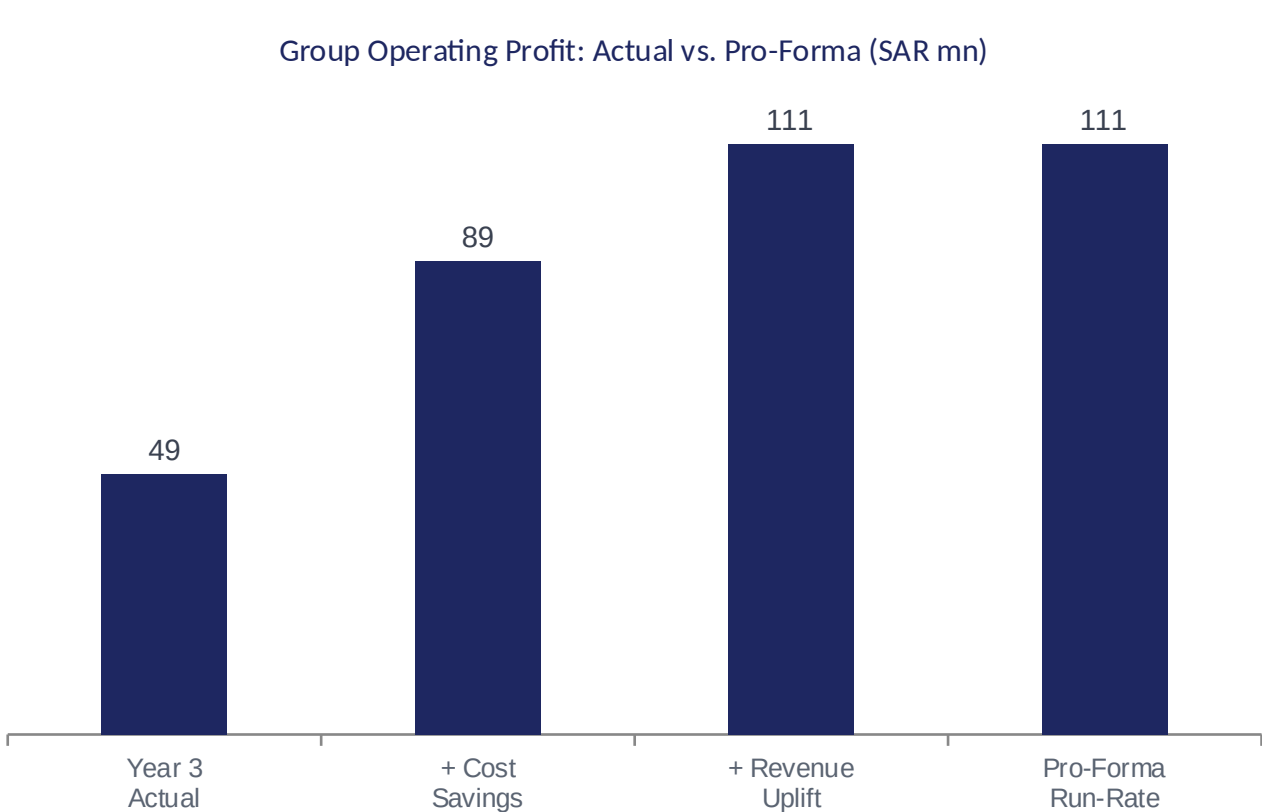
3

## Structural

18–36 months

- Complete East's shift to a variable, demand-matched staffing model
- Re-baseline Central's governance and decision rights group-wide
- Re-assess a second growth site only once East's model is proven

# The Fix Is Worth ~SAR 62mn a Year, Run-Rate



**SAR 40mn**

Annual cost savings run-rate — Central overhead, East staffing model, group procurement

**SAR 22mn**

Annual revenue uplift run-rate — occupancy & throughput gains at Central and East

**16% margin**

Pro-forma group operating margin — back near Year-1 levels (16.1%)

*These are B2O fixes, not new-revenue bets — they close the gap between how each hospital is priced and how it is run.*

# Fix the Operating Model at Two Hospitals — Before Chasing More Growth

The business is underperforming because **Al-Noor Central** and **Al-Noor East** are running operating models that don't match their **business models** — and we recommend **delaying** Central's overhead and rebuilding East's staffing model, using Premium Ortho & Spine's operation as the group playbook.

**SAR 62mn/yr**

Run-rate profit impact

**~16%**

Pro-forma margin, restored

**3 phases**

Quick wins → medium-term → structural

**2 of 3**

Hospitals need operating-model fixes

*NEXT: confirm the Central delaying scope and East staffing plan with hospital directors; stand up a group transformation office to track the roadmap against the Sheet-3 savings targets.*